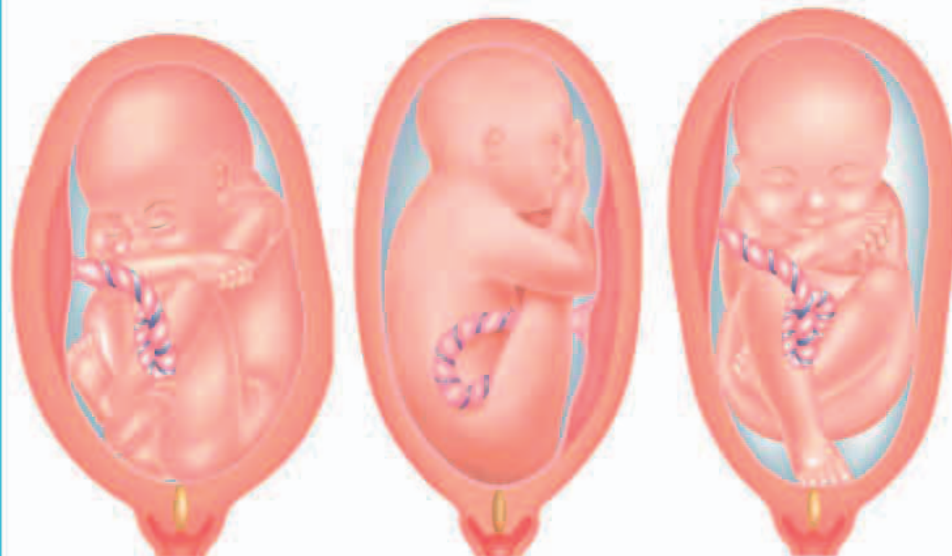


Q I'm 36 weeks pregnant and my baby is breech. Should I worry?

There's still time for your baby to turn—some babies change position up until days before birth—but your doctor will discuss your options with you.

Most babies have turned into a head-down, vertex position by 34 weeks pregnant, but it's also perfectly normal for a baby to move and swing around a few days before birth, and even during labor itself. It's an evolving process. If the baby hasn't turned by 36 weeks, your doctor will offer to try manually turning your baby with a technique called external cephalic version (ECV).

If successful, this can help avoid a cesarean section. Your doctor should spend time with you to discuss the pros and cons of vaginal delivery versus cesarean section so that you can make a fully informed decision in the event that your baby stays in a breech position. Not every situation will be the same, partly because there are three possible breech positions (shown below).



Complete (flexed) breech In this bottom-downward position the baby's legs are flexed and he is curled up, knees and chin to chest, and with ankles crossed.

Frank (extended) breech Both the baby's legs are extended so that his feet stick upward, and his bottom presents first. He might be hugging his knees.

Footling breech This baby presents his foot first: both knees are bent, but one foot is extended downward, pressing against the cervix.

Q Can I turn a breech baby myself, or will a doctor have to do it?

If you are past 36 weeks pregnant, you should be offered an external cephalic version (ECV).

This noninvasive technique involves a doctor pressing hard on your abdomen to try to push the baby round into the cephalic (head-down) position. It's a perfectly safe procedure for both of you, and works in around half of all cases, but it may feel a bit uncomfortable at the time. If it works, ECV greatly lowers the risk of cesarean section. If you want to help things along at home, some moms report that adopting

certain positions to "tip" the baby out of the pelvis can help give it more room to move. You could try either kneeling forward on the floor with one cheek resting on the floor and your bottom in the air, or lying on your back with a pillow beneath your hips to raise them, knees bent, feet flat on the floor. While there is no research evidence to suggest that these techniques will necessarily work, they won't harm your baby either. Just be careful you don't hurt your back or neck in the process. You can also try moxibustion, which advocates of traditional Chinese medicine believe can turn the baby. Acupuncture is also a popular therapy to try.

Q If my baby isn't lying head down, will I definitely have to have a cesarean section?

Although cesarean sections are often considered the safest option for breech babies, sometimes a vaginal birth is possible. If your baby presents bottom first, is of average size, and you have an average-sized pelvis, your doctor may be happy to allow a vaginal delivery, if that is your preference. If the baby presents with foot or knee first, cesarean section is usually advised as the safer option.

Only around 3.5 percent of births are breech (bottom downward). Your doctor may try to turn your baby into a vertical position using ECV (see below, left), during the last few weeks of your pregnancy if he himself isn't showing signs of making the turn. If your baby is transverse (lying across your uterus), however, unless the baby can be turned manually, you will probably need to have a cesarean section.

Q Will I be able to have a normal experience of labor if my baby is in a breech position?

Yes. The early stage of labor is more or less the same for both vertex and breech births, but you're likely to be monitored more frequently if your baby is breech. You can still move around, but your doctor may need you to remain attached to a fetal heart monitor, which would limit your movements a bit. Ask if there are any portable methods for monitoring the baby, and make a decision based on your doctor's advice. Keep emptying your bladder during your labor—every two hours or so—in order to give your baby as much space as possible to move into a good position for birth.

Remember to monitor your baby's movements in the uterus. Reporting changes in the pattern of behavior to your doctor immediately could prevent a stillbirth.